

METROPOLITAN GENERAL HOSPITAL

2100 Healing Way, Boston, MA 02115 | (617) 555-0800 | www.metrogen.org

DISCHARGE SUMMARY

PATIENT INFORMATION

Patient Name:	Robert James Caldwell	DOB:	March 14, 1968 (Age 57)
MRN:	MRN-0045821	Gender:	Male
Admission Date:	April 8, 2025	Discharge Date:	April 14, 2025 (6 days)
Attending Physician:	Dr. Angela Forsythe, MD — Cardiology	Insurance:	BlueCross BlueShield #BCB-779-00234

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Acute ST-Elevation Myocardial Infarction (STEMI) — Anterior Wall	I21.09	Resolved
Type 2 Diabetes Mellitus	E11.9	Chronic / Pre-existing
Hypertension, Essential	I10	Chronic / Pre-existing
Acute Kidney Injury, Stage 1	N17.9	Resolved

HOSPITAL COURSE

Mr. Caldwell, a 57-year-old male with a known history of Type 2 Diabetes and Hypertension, presented to the Emergency Department on April 8, 2025 via ambulance with a 2-hour history of severe substernal chest pain radiating to the left arm, diaphoresis, and nausea. Initial ECG demonstrated ST-segment elevation in leads V1–V4 consistent with anterior STEMI. Troponin-I on arrival was critically elevated at 18.4 ng/mL. The patient was taken emergently to the cardiac catheterization laboratory where coronary angiography revealed 95% occlusion of the left anterior descending (LAD) artery. Successful percutaneous coronary intervention (PCI) was performed with deployment of a drug-eluting stent (Xience Skypoint 3.0 x 28mm). Post-procedural TIMI flow was grade 3. The patient was transferred to the Cardiac Care Unit (CCU) for monitoring. Serial echocardiography on day 2 showed an ejection fraction of 42%, mild anterior wall hypokinesis. Mild acute kidney injury (Creatinine 1.8 mg/dL) was noted, attributed to contrast nephropathy; this resolved with IV hydration by day 4. Blood glucose remained elevated throughout admission and endocrinology was consulted to optimize insulin regimen. The patient remained hemodynamically stable throughout and was discharged on day 6 in stable condition with close outpatient follow-up arranged.

VITALS AT DISCHARGE

BP	HR	SpO2	Temp	RR	Weight
128/78 mmHg	72 bpm	98% (RA)	98.4°F	16/min	91 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Aspirin	81 mg	Once daily	Indefinite
Clopidogrel (Plavix)	75 mg	Once daily	12 months
Atorvastatin	80 mg	Once at bedtime	Indefinite
Metoprolol Succinate	50 mg	Once daily	Indefinite
Lisinopril	10 mg	Once daily	Indefinite
Insulin Glargine (Lantus)	22 units	Once daily at bedtime	Ongoing — follow with Endo

Metformin	500 mg	Twice daily with meals	Ongoing
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FOLLOW-UP PLAN

Cardiology:	Dr. Forsythe — 1 week post-discharge. Repeat Echo at 6 weeks.
Endocrinology:	Dr. Patel — 2 weeks post-discharge for diabetes management.
Primary Care:	Dr. Kim — 4 weeks. Renal function panel to confirm AKI resolution.
Cardiac Rehab:	Enrollment arranged — starts May 5, 2025 at MetroGen Rehab Center.
Labs:	BMP, HbA1c, Lipid Panel — 2 weeks post-discharge.

Attending Physician Signature: _____
Dr. Angela Forsythe, MD FACC | Cardiology
License #: MA-MD-44521 | Date: April 14, 2025

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Document ID: DS-2025-0045821-04
Generated: April 14, 2025 at 15:42 EST